

● JESS MOVE

Movement that fits the working day.

Small Moves. Powerful Change.

A movement and food-intelligence platform for your workforce — built so an employer can run a programme without ever seeing one employee.

Design partner programme · 2026

● WHERE WE ACTUALLY ARE

No customers yet. That is the offer.

Most vendors you meet will show you logos and a case study. We have neither, and saying so is the point — you are not buying a finished programme, you are shaping one. What exists is the platform, built and tested; what does not exist is a track record.

942

automated tests in
the repository today

8

people needed before
a figure is reported

0

organisations that have
run a cohort so far

● **Built and tested**

Safeguarding and privacy rules enforced in the database, not the interface.

● **Not yet proven in production**

No organisation has run a cohort. Any figure we showed you would be invented.

● **What we want**

Three to five design partners. Real employees, real calendars, real feedback.

● THE PROBLEM YOU ALREADY HAVE

Your people sit down at nine and stand up at five.

The programme nobody opens

A wellbeing app is bought, announced, used enthusiastically for two weeks, and then quietly abandoned. Renewal becomes a conversation about why.

The programme nobody trusts

Anything that can show an individual's activity to their employer will eventually be asked to. Your people know that, and behave accordingly.

The programme that excludes

Step challenges reward the people who were already fit. The employee who most needs it is the one who cannot take part.

We have built for the second and third of these deliberately, because they are the ones a platform can actually decide.

- THE PROCUREMENT QUESTION

Can an employer see one named person's activity?

No.

Not restricted. Not audited. Not available to a super-admin.

OrganisationReport

`apps/backend/src/groups/groups.logic.ts` — the type your engineers can read in five minutes.

Why that answer holds

- The organisation report type has no field for a person. It carries a size, a participation percentage, an active count, a median and a suppression flag — so a client asking for a name does not compile.
- A permission is a setting, and settings change when somebody senior asks. Because the capability was never built, adding one takes a schema change, a migration and a code review.
- A cohort figure needs at least 8 contributing people. Below that the field is null, not a rounded stand-in.

● NOBODY IS EXCLUDED BY DESIGN

Five variants, or it does not ship.

Every movement in the library exists as five independently authored variants. Not one version with things removed — a good seated movement uses the chair. The publishing gate refuses anything with fewer. There is no force-publish flag and no admin bypass.

1

Standing

2

Seated

3

Chair-supported

4

Bed / Recliner

5

Single-limb /
Adaptive

Four of the six age bands are working age — 18–39, 40–64 and 65–79, plus apprentices under 18. The band is derived, not chosen, and it changes the mechanics, the daily prompt cap and the clinical guardrails. It is not one programme with the age swapped.

● WHY ENGAGEMENT DOES NOT DECAY

It decides when to say nothing.

A reminder delivered while somebody is driving is not a neutral event. It costs attention, then trust, and eventually the notification is switched off — at which point the programme is finished however good the recommendation was. So a block is recorded as a successful decision, not a failed delivery.

What silences a prompt

- Driving, from motion state rather than location
- On a call, in a lesson, or notifications turned off
- The daily cap, which differs by age mode
- Quiet hours, the sleep window, or an active cooldown
- A clinical rest flag, a safeguarding hold or a bereavement hold

The calendar, read without being read

Start time, end time, busy or free, attendee count, whether it recurs.

The title is never transmitted, never logged and never sent to a model. It costs us accuracy — a one-to-one and a board review look identical to us — and we accept the worse recommendation.

● WHAT YOU ACTUALLY RECEIVE

Enough to run a programme. Not enough to manage a person.

Today

The cohort report returns

- Participation rate across the cohort
- How many people were active
- Median days moved in the period
- Whether the answer was suppressed, and the floor

Not built yet

Built with a design partner

- A trend series against a baseline
- Sedentary-risk distribution by cohort
- A return-on-investment model, assumptions shown
- Single sign-on and directory sync

Below eight contributing people every figure is null, not rounded — for you as well as for everyone else. An eleven-person company cannot have anonymous reporting however it is sold.

● FOR YOUR SECURITY REVIEW

A claim you can call is worth more than a claim you can read.

Checkable from a terminal, without an account and without us in the room. The assurance endpoint lists 32 controls — 21 enforced in code, 7 implemented, 4 still open.

GET `/api/assurance`

Every control, its status, and the gaps — listed at the top, not buried.

GET `/api/movements/gate`

The publishing contract: five variants, a cue set per age mode, no override.

POST `/api/body/assess`

Send a twelve-year-old with consent set true. It returns metrics: null.

GET `/api/health`

A live check that opens a database connection, never a stored badge.

Four things enforced. Four things open.

Enforced in code and schema

- A declared condition is stored as a catalogue identifier. No severity, no dates, no medication, no free text.
- Deleting an account cascades in the database from the user row — a guarantee, not a callback somebody has to remember.
- Health data never reaches an organisation view at any cohort size. A test asserts that no module outside the conditions service touches that table.
- Analytics identifiers are hashed on arrival with a salt regenerated daily, and never outlive the request.

Open, and named before you ask

- No external WCAG 2.2 AA audit and no VPAT. Accessibility is designed for and tested in places, not certified.
- Age is self-declared. Ofcom's position is that self-declaration is not highly effective age assurance.
- A member can read their whole record through their own endpoints, but there is no one-click export archive yet.
- The Clinical Safety Officer appointment record is incomplete against DCB0129, and the hazard log says so itself.

The right-hand column is not a list we were pushed into. It is what GET /api/assurance returns when anybody calls it, with the gaps lifted to the top of the response rather than left to be found area by area.

From £2 per employee per month.

from £2

per seat, per month

The floor, not a teaser. Up to £5 depending on sector, seat count and support level. Organisation deals are contracts, so the figure is agreed rather than picked at a checkout.

Minimum 10 seats

And a minimum annual contract.

Challenge builder and campaigns

Branded to your organisation.

Setup and integration support

Included in every contract.

Single sign-on and directory sync

Contracted, not yet built. Delivered inside the pilot.

Design partners are priced at the floor for the first year, in exchange for feedback and a reference once there is something real to reference.

● THE ASK

A twelve-week pilot, one team, no procurement epic.

1

Week 0

One team, 10-100 people. Invited by email, and a named sponsor on your side and ours.

2

Weeks 1-8

People use it. We watch the held rate and completion, not the vanity numbers.

3

Weeks 9-12

Cohort report against baseline. Honest review — including whether it did nothing.

If the pilot shows it did not work, we would rather know at week twelve than sell you a second year. That is the whole basis of the programme.



Two minutes is the whole ask.

Of an employee's day, and of yours to decide whether a twelve-week pilot is worth running.

Next step

jessmove.com/industries

A thirty-minute call, and we will run the verification endpoints live if your security team wants to watch.

Jess Move does not diagnose any condition and never contacts emergency services. In the UK, call 999 for an emergency and 111 for urgent health advice.